



### Clinical Case Quiz

A 2-month-old female infant presents via telephone appointment at 10:45 a.m.

#### History

- 4:30 a.m.: Infant was “kicking around” as if in pain, then calmed and fed well.
- 7:30 a.m.: Became very fussy, noted to have non-bloody diarrhea. Shortly afterward, vomited twice (non-projectile, non-bloody, non-bilious).

**Review of Systems:** No fever, cough, congestion, or runny nose. No prior reflux or vomiting history

**Social History:** No current sick contacts. Two weeks ago, family members had “stomach flu.”

What is the next best step during this telephone visit?

- Provide supportive care measures and return precautions for gastroenteritis.
- Recommend that the mother call 833-KP-4-CARE for an in-person appointment.
- Direct the patient to the emergency department.
- Obtain a “virtual physical exam” to better assess and triage.

The answer is d.



### Discussion: Choosing the Best Path for Care

Obtaining a virtual physical exam is vital. In telehealth, we lack the ability to physically examine the patient. In the above scenario, we need more information. A “virtual physical exam” involves asking specific historical and patient-guided exam questions to more thoroughly assess and triage. The key determination is **sick** or **NOT sick**, which then guides triage and/or treatment.

### Case Findings and the Next Step



#### Virtual Physical Exam Case Findings

- Parent reports that the abdomen is soft and non-tender to palpation.
- Mother described the infant as **very** pale and sick-appearing, with frequent head shaking movements, which mother interpreted as pain.

#### Video Visit Conversation

This prompted a conversion to video visit ([a guide to converting may be found here](#)) to verify parental findings, and the infant was indeed very pale and ill-appearing with a weak cry.

What is the now the most appropriate next step?

- Virtual care: Provide supportive care measures and return precautions for gastroenteritis.
- Recommend that the mother call 833-KP-4-CARE for an in-person appointment.
- Direct the patient to the emergency department.

The answer is c.

### Discussion: The Next Best Step and Key Takeaways

The answer is C. The infant's appearance (very pale, weak cry, ill-appearing) indicated a high-risk presentation. In virtual care, we do not always need to establish the diagnosis immediately, but we **must** always triage appropriately.

#### ED Course

- Rule out sepsis: work-up was normal.
- Infant had recurrent vomiting (progressing to bilious) and passed bloody stool, which ultimately led to an abdominal ultrasound.
- Ultrasound: Intussusception.
- Managed with contrast enema reduction → successful.
- Discharged in good condition after overnight observation.



#### Key Learning Points

1. A “virtual physical exam” is critical. In the absence of an in-person exam, clinicians must use structured questions and patient-guided exams, when appropriate.
2. We must always determine if the patient is **sick** or **not sick**.
3. Not all vomiting and diarrhea = gastroenteritis. Differential in this case included intussusception, pyloric stenosis, malrotation with volvulus, and sepsis.
4. We don't always need an immediate diagnosis virtually, but we must **always** recognize when escalation is required.

#### Example SmartPhrases for Virtual Physical Exam

##### General Appearance

- “The patient sounded well and spoke in complete sentences without difficulty.”
- “The patient sounded playful in the background.”
- “Parent states he is active and playful still.”
- “Parent states he is **NOT** very ill-appearing.”

##### Respiratory

- “I had the child sing the ABCs over the phone, and he did so without any significant pauses, tachypnea or shortness of breath.”

##### GI

- “Abdomen is soft and non-tender to parental palpation.”
- “Patient can jump without pain.”

#### Clinical Takeaway

A thorough history plus a structured virtual physical exam empowers clinicians to safely triage patients, distinguishing those who can be managed virtually from those needing escalation.